

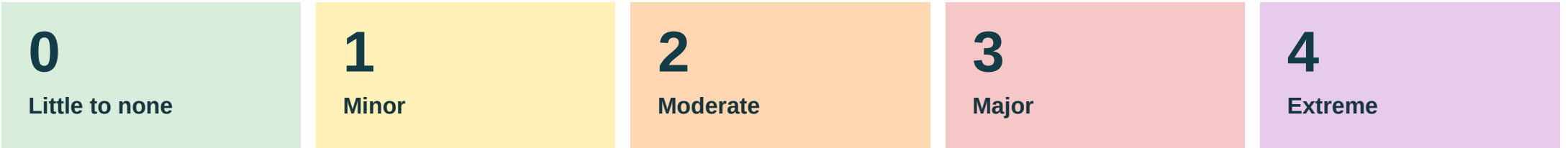
Two separate inputs. One clear assessment.

H describes forecast heat. V describes patient vulnerability. Never add H and V together.

H

Environmental heat severity | 0-4

Use the official daily NWS HeatRisk value for the patient location and forecast date. [1]



Missing forecast = unknown, not 0. HeatRisk is not a personal probability of illness.

V

Patient vulnerability | 0-10

Five domains, each worth 0-2 points. Apply these conditions before adding the domains.

A

Clinical susceptibility
+1 each: age 65+; heart failure; chronic kidney disease; diabetes; previous heat illness.

Add applicable factors, then cap at 2. No factors = 0. [3, 4, 5]

B

Medication considerations
0: reviewed list has no heat-relevant drugs. 1: at least one heat-relevant drug.

2: diuretic + ACE inhibitor, ARB or ARNI. Take highest only. [2]

C

Cooling access
0: reliable home cooling. 1: alternative cooling only.

2: neither home nor accessible alternative cooling. Choose one. [6]

D

Heat exposure
0: no expected exposure. 1: exposure with cooling breaks, without strenuous activity.

2: exposure without cooling breaks OR strenuous activity in heat. [5]

E

Support and assistance
+1: no reliable check-in person. +1: needs help reaching cooling or following the plan.

Add both factors. Actual support matters; living alone alone adds no point. [6]

V = A + B + C + D + E

Maximum 10 points. Not a percentage.

From the scores to preventive outreach

Use H and V as separate axes. The matrix below is a provisional project policy.

1 Match H and V in the outreach matrix

Official HeatRisk (H)	V = 0-2	V = 3-5	V = 6-10
0 Little to none	Routine	Routine	Routine
1 Minor	Routine	Targeted	Targeted
2 Moderate	Targeted	Targeted	Priority
3 Major	Targeted	Priority	Priority
4 Extreme	Priority	Priority	Priority

Routine: maintain a heat plan. **Targeted:** personalized prevention and support checks.
Priority: flag for care-team review before heat exposure; this is not an emergency diagnosis.

2 Worked example | synthetic patient

Age 72 with diabetes; diuretic + ACE inhibitor; alternative cooling only; can avoid heat exposure; no check-in person and no assistance needs.

Clinical	Medication	Cooling	Exposure	Support	TOTAL V
2	2	1	0	1	6 / 10

H = 2 AND V band 6-10 => PRIORITY OUTREACH

Use the two values to find the matrix cell. H and V are never summed.

Before scoring: route symptoms through a separate clinician-reviewed pathway. Unknown inputs remain unknown; show a score range and request information if priority could change. Do not recommend medication changes.

EVIDENCE AND IMPLEMENTATION NOTES

- 1 NWS HeatRisk
- 2 CDC medication guidance
- 3 CDC older adults
- 4 CDC clinical reference
- 5 NIOSH heat stress
- 6 Bouchama et al., 2007

Medication scoring requires a clinician/pharmacist-reviewed, non-exhaustive heat-relevant drug list. Cooling access includes opening hours, transport, affordability and usability. Exposure includes hot indoor work. Source evidence supports factor selection, not the assigned weights or matrix thresholds.